

MONROE COUNTY FIRE RESCUE — TRAUMA STAR

PRACTICE CHART — FICTITIOUS PATIENT — FOR QI TRAINING ONLY — NOT A MEDICAL RECORD

Practice Chart 10: IFT, Respiratory Failure (BiPAP) (target: ~80% complete)

Date / OCA	2026-07-27 / PC-2010	Aircraft	Trauma Star South
Mission	Interfacility (Air)	Chief Complaint	COPD exacerbation, respiratory failure
Pilot	R. Diaz	Nurse / Medic	M. Chu, RN / T. Beck, Paramedic
Pickup / Dropoff	Mariners Hospital	Kendall Regional ICU	
Dispatch / Wheels Up	1642 / 1649	Land / Depart	1704 / 1719

Flight Narrative

Requesting agency: Mariners Hospital ED. Sending physician Dr. Neil Park, MD; accepting physician Dr. Sara Otto, MD, Kendall Regional ICU. Diagnosis: COPD exacerbation with acute respiratory failure on BiPAP, a time-sensitive emergency. Sending facility lacks ICU capability, not available at the pickup location.

HPI: 72-year-old with three days of worsening dyspnea; events leading to transfer: rising CO2 despite treatment. Prior to arrival: duonebs, solumedrol 125 mg IV. Infusions running prior to assessment: none; BiPAP 12/5 FiO2 50% in progress. Labs: pH 7.28, pCO2 68 on blood gas; chest x-ray hyperinflated, remaining labs pending.

Rotor-wing air transport chosen: ground transport time approximately 90 minutes per Google Maps versus 23 minutes by air; clinically significant for this patient with ventilatory failure. Requires continuous monitoring and titration beyond the scope of available ground providers. Transported to Kendall Regional, whose ICU capability meets the patient's needs.

1706 BP 142/86 HR 108 RR 26 SpO2 91% on BiPAP FiO2 50% — mild hypoxia noted; FiO2 increased to 60% at 1708, reassessed 1716 SpO2 95%, work of breathing improved. Glucose 188. Pain 1/10 at initial assessment and 1/10 at handoff. 1716 BP 138/84 HR 102 RR 22 SpO2 95%. 1726 BP 136/82 HR 98 RR 20 SpO2 96%; respiratory status stabilizing with ongoing BiPAP therapy.

Max altitude 1500 ft. Radio report given to Kendall Regional at 1722; ICU bed confirmed and respiratory therapy notified. Care transferred to ICU team; patient remains at high risk of ventilatory deterioration without continuous critical care monitoring.

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